Utah Department of
Health & Human Services
Licensing & Background Checks

Inspection Checklist

This tool is a preliminary inspection checklist designed to ensure consistency across all inspections. ***(Revised Jan 2026)***

Provider Name:	Weber Recovery Center, LLC (RT, SD, OT)	**Facility ID:**	F22-95883	**Phone Number:**	(385) 445-4553	**Notes**
Site Name or Address:	2740 Pennsylvania Ave Ogden, UT, 84401			**Email Address:**	patriciac@weberrecovery.com; brian.m@weberrecovery.com	
Approved Capacity:	RT:38 SD:5	**# of Present Residents\Clients:**	RT: 19 SD: 0			

Please review the following items prior to the inspection: **(Mark with a check mark if completed and make any necessary notes)**			**Please review the following items during the inspection:** **(Mark with a check mark if completed and make any necessary notes)**		
☑	**Current backgrounds in DACS**		☑	**Any active rule variances**	No current variances
☑	**Current staff roster collected**		☑	**Introduce yourself and any DHHS staff**	
☑	**Any license restrictions or conditions**	No restrictions/conditions	☑	**Staff Interviews**	
☑	**Any needed rule variances**	Not requesting a variance	☑	**Clients Interviews**	

Inspection Information:

- These are initial observations and do not constitute a final inspection report. The Licensor will email you this inspection checklist after the inspection is completed if requested. An official inspection report will be sent to you once this inspection has been approved by OL management.

Inspection Type:	Announced Annual	**Date:**	2/11/2026	**Time Started On-site:**	10:00 AM	**Time Ended On-site:**	1:30 PM
Number of Items Under Review:		4	**Name of Individual Informed of this Inspection:**		Patricia Cardwell		
Licensor(s) Conducting this Inspection:		Mary Bokinskie; Josilyn Bertrand			**OL Staff Observing Inspection:**		

Adult Residential General Provisions - Inspection Checklist

(Revised Jan 2026)

R = Reviewed.
UR = Under Review. Under Review indicates evidence of potential noncompliance.
NA = Not Assessed during this inspection.

New and Renewal Licensing Procedures	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-3(1) Until a license or certificate is approved by OL, an applicant or provider may not: (a) accept any fee; (b) enter into any agreement to provide a client service; or (c) provide any client service.	☑	☐	☐	☐	☐	
R380-600-3(3) An applicant or a provider shall permit the office to have immediate, unrestricted access to: (a) any unaltered on and off-site program or facility and client records; (b) each client who independently consents to speak to OL staff; (c) each site subject to licensing or certification; and (d) each staff member.	☑	☐	☐	☐	☐	
R380-600-3(4) A provider may not permit a staff or client to threaten, verbally or physically abuse, or use violence of any kind while interacting with a representative of the department.	☑	☐	☐	☐	☐	
R380-600-3(15) The license or certificate holder shall adhere to any individualized parameter on a program or facility license or certificate to promote the health, safety, and welfare of any client. Parameters may include: (a) adequate square footage to determine capacity; (b) an admission or placement restriction; or (c) an age restriction.	☑	☐	☐	☐	☐	
R380-600-3(18) Unless previously approved by OL to provide services before receiving a license or certificate for special circumstances, a provider must submit an application, any required fee, and obtain a new or a renewed license or certificate before providing any service that requires a license or certificate.	☐	☐	☑	☐	☐	Program did not provide services before being licensed.
R380-600-3(25) The provider shall post their current license or certificate, except in a foster home, on the premises in a place readily visible and accessible to the public.	☑	☐	☐	☐	☐	
R501-14-3(1) A provider representative shall ensure that an applicant for an initial background check completes the required application fields and disclosure statements to authorize OBP's continual monitoring of the applicant's fingerprints and applicable state registries.	☑	☐	☐	☐	☐	
R501-14-4(3)(a) The provider representative shall keep the program's roster and employee information current in DACS. **(b)** The provider representative shall check the roster at least monthly to verify employee information and the employment of employees due for a renewal review.	☑	☐	☐	☐	☐	
R501-14-5(2)(a) The provider representative shall submit a background check application for each applicant for an initial background check no later than two weeks from the date the applicant becomes associated with the licensee, certification, or contract. **(b)(i)** The provider representative shall ensure an applicant is directly supervised until OBP issues a conditional or eligible clearance determination. **(ii)** The provider representative shall document how the applicant remains supervised for the entirety of the applicant's supervised employment term before receiving a clearance determination.	☑	☐	☐	☐	☐	
R501-14-5(5)(a) The provider representative may not allow an applicant whose background check application is denied to have any supervised or unsupervised direct access to clients unless: **(i)** OBP approves a subsequent application; or **(ii)** the denial is overturned in an administrative hearing or by the OBP director. **(b)** The provider representative shall ensure an applicant initiating an appeal of a denied application works under direct supervision until OBP issues a determination regarding the appeal.	☑	☐	☐	☐	☐	

Variances	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-6(5) The provider shall sign the approved variance and comply with the terms of the written variance, including any conditions or modifications contained within the approved written variance.	☑	☐	☐	☐	☐	No current variances

Inspection and Investigation Process	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-7(16) When a critical incident occurs under the direct responsibility and supervision of the program or facility, the provider shall: (a) submit a report of the critical incident to OL in a format required by OL within one business day of the critical incident occurrence; (b) additionally ensure any allegation of an incident of abuse, neglect, or exploitation of a client is reported to DCFS for a minor client or APS for an adult client and law enforcement within 24 hours; (c) notify the parent or legal guardian of each involved client within a 24-hour period from the time of the incident; (d) if the critical incident involves any client in the custody of the department or under contract with the department, notify the involved department division immediately; and (e) collect, maintain, and submit original witness statements and supporting documentation, including video footage if available, regarding each critical incident to OL upon request.	☑	☐	☐	☐	☐	

Program Policies, Procedures, and Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-4(2) The licensee shall develop, implement, and comply with safe practices that: (a) ensure client health and safety; (b) ensure the needs of the client population served are met; (c) ensure that none of the program practices conflict with any administrative rule or statute before implementation; and (d) inform staff of how to manage any unique circumstances regarding the specific site's physical facility, supervision, community safety, and mixing populations.	☑	☐	☐	☐	☐	
R501-1-4(3) The licensee shall submit any change to an office approved policy or curriculum to the office for approval before implementing the proposed change.	☑	☐	☐	☐	☐	

Residential Programs Additional Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-5(1) A licensee that manages, stores, or administers client medications shall develop and ensure compliance with the following medication management safe practices: (a) inform staff and clients of program and client responsibility for medication including storage and administration of medications on-site and, as applicable, when staff and clients are offsite in program related activities; (b) if applicable, inform staff and clients of the medication self-administration process; (c) if storing and administering medications, train staff to administer medication and the process to be followed; (d) how staff record medication dosages according to prescriptions; (e) how staff monitor for and record effects and side effects of medications; and (f) how staff log doses and record and report medication errors.	☑	☐	☐	☐	☐	
R501-1-5(2) The licensee shall ensure the care, vaccination, licensure, and maintenance of any animals on-site to include: (a) assessment of pet allergies for any clients interacting with animals in the program; (b) maintenance of required examinations, registrations, and vaccinations; and (c) supervision of clients in the presence of animals.	☐	☐	☑	☐	☐	No animals on site.

R501-1-5(4) The licensee shall ensure that a ratio of one staff to one client during transports is only utilized when the program has conducted a safety assessment that indicates that client and staff safety is reasonably assured.	☑	☐	☐	☐	☐	
Program Administrative and Direct Service Requirements	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-1-6(1) The licensee shall clearly identify services to the office, public, potential client, parent, or guardian regarding: (a) current and accurate contact information; (b) the complaint reporting and resolution process; (c) a description of each service provided; (d) each program requirement and expectation; (e) eligibility criteria outlining behavior, diagnosis, situation, population, and age that can be safely served, including: (i) an outline of the behaviors and presenting issues that would be reason for discharge or exclusion from the program; and (ii) a statement that the program may not take placement of a child whose needs exceed the scope or ability of the program to reasonably manage; (f) each cost, fee, and expense for a service and refund policy; and (g) identification of each non-clinical, extracurricular, or supplemental service offered or referred.	☑	☐	☐	☐	☐	
R501-1-6(2) The licensee shall post the following in conspicuous places where each visitor, staff, and client may view: (a) abuse reporting laws as described in Sections 80-2-609 and 26B-6-205; (b) civil rights notice; (c) Americans with Disabilities Act notice; (e) any office notice of agency action; (f) a client rights poster in a residential setting except in a foster home or where prohibited by Settings Final Rule; and (g) department code of conduct poster.	☑	☐	☐	☐	☐	
R501-1-6(3) The licensee shall maintain compliance with or documentation of an exemption from any of the following requirements: (a) a food handler permit for any person preparing meals for any other person; (b) capacity determinations that include each staff and client on premises and may not exceed the capacity limits placed by local authorities; (c) fire clearance, if conducted separately from a business license; (d) licensure and registration of any vehicles used to transport clients.	☑	☐	☐	☐	☐	
R501-1-6(5) The licensee shall maintain and make the following available to the department upon request: (b) general liability insurance; (c) professional liability insurance; (d) vehicle insurance; (e) fire insurance; and (f) additional insurance as required to cover each program activity.	☑	☐	☐	☐	☐	
R501-1-6(6) The licensee shall ensure: (b) current staff and client lists are available at each licensed site; (c) the organizational and governance structure of the program is available to the department upon request and includes: (i) line of authority and responsibility; (ii) a job description, including each duty and qualification for each job title; and (iii) notification to the office of any program changes as described in Section R380-600-3; (d) a quality improvement plan that is implemented and followed and that incorporates, at a minimum, client and staff grievances, feedback, trends in licensing noncompliance and incident reports; (e) the licensee provides an interpreter or refers each client to appropriate resources as necessary to communicate with the client; (f) the licensee maintains an opioid overdose reversal kit on-site with on duty staff trained in its use if the licensee is serving, or is likely to	☑	☐	☐	☐	☐	

R501-1-6(9) The licensee shall ensure clinical and medical staff are licensed or certified in good standing and any unlicensed staff are appropriately supervised	☑	☐	☐	☐	☐	
Residential Program Additional Administration and Direct Services Requirements	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-1-7(1) A residential program licensee shall additionally: (a) ensure each staff shift list remains current and available to the office upon request; (b) ensure that each shift documents any illness, injury or critical incident and passes it on to the next shift and administration; (d) ensure access to a medical clinic or a medical professional familiar with the program and population served; and (e) provide a separate space for clients who are showing symptoms of an infectious disease.	☑	☐	☐	☐	☐	
R501-1-7(6)(a) A residential program licensee, excluding a residential treatment program, may allow for client independence and responsibility for their own supplies, food, laundry, or transportation by outlining in writing resources and responsibility for the provision of these items. (b) Each residential program licensee shall assist clients on a limited basis if they are temporarily unable to provide the items or services listed in Subsection R501-1-7(6)(a) for themselves.	☐	☐	☑	☐	☐	Clients are not responsible for finding a laundromat or food; the program supplies all these items.
Program Physical Facilities and Safety	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-1-8(1) The licensee shall ensure: (a) the appearance & cleanliness of the building/grounds are maintained & free from health/fire hazards; (b) any appliances, plumbing, electrical, HVAC, and furnishings are maintained in operating order and in a clean and safe condition; (c) fire drills in non-outpatient programs are conducted at least quarterly and documented, including feedback regarding response time and process; (d) a phone that can be used to call 911 is always available on-site when clients are present; (e) bathroom facilities for staff and clients allow for individual privacy and afford reasonable accommodation based on gender identity; (f) each bathroom is properly equipped with toilet paper, paper towels or a dryer, and soap; (g) each bathroom is ventilated by mechanical means or equipped with a window that opens; (h) non-prescription medication, if stored on-site, is stored in original manufacturer's packaging together with the manufacturer's directions and warnings; and (i) prescription medication, if stored on-site, is stored in original pharmacy packaging or individual pharmacy bubble pack together with the pharmacy label, directions, and warnings.	☑	☐	☐	☐	☐	
R501-1-8(2) The licensee shall accommodate a client with physical disabilities as needed or appropriately refer to comparable services.	☑	☐	☐	☐	☐	
R501-1-8(3) The licensee shall maintain medication and potentially hazardous items on-site lawfully, responsibly, and with consideration of the safety and risk level of the population served to include locked storage for each medication and hazardous chemical that is not in active use.	☑	☐	☐	☐	☐	
R501-1-8(4) The licensee shall maintain a first aid kit	☑	☐	☐	☐	☐	
Residential Program Additional Facilities and Safety Requirements	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**

R501-1-9(1) A residential licensee shall ensure: (a) designated space is available for records, administrative work, & confidential phone calls for clients; (c) live-in staff have dedicated bedrooms & bathrooms separate from client use; (d) each bedroom designated for a client is comparable to other similarly utilized bedrooms with similar access, location, space, finishings, and furnishings; (e) clients are not locked in bedrooms; (f) a mirror or safety mirror is secured to each bathroom wall at a convenient height; (g) each bathroom is placed to allow access to each client without disturbing any other client during sleeping hours; (h) each bath or shower allows for individual privacy; (i) each client is supplied with hygiene supplies; (j) each sleeping area has a source of natural light and is ventilated by mechanical means or is equipped with a window that opens; (k) each client has a similar solid type of bed or sleeping equipment to any other client in the program; (l) each client is allowed to decorate & personalize their bedroom, while maintaining respect for other residents and property; (m) there are separate containers for soiled & clean laundry, if the program provides common laundry for towels, bedding or clothing; (n) bedding & towels are laundered weekly & after each client is discharged; (o) equipment and supplies for washing & drying laundry are provided, if the program permits clients to do their own laundry; (p) there is at least 60 sq ft per person in a multiple occupancy bedroom and 80 sq ft in a single occupant bedroom.	☑	☐	☐	☐	☐	
R501-1-9(2) A residential program licensee serving individuals with disabilities shall house no more than two clients in each bedroom.	☑	☐	☐	☐	☐	
R501-1-9(3) The licensee utilizing seclusion rooms shall ensure the following: (a) seclusion rooms measure a minimum of 75 sq ft and have a minimum ceiling height of 7 ft with no equipment, hardware or furnishings that obstruct staff's view of the client or present a hazard; (b) a seclusion room shall have either natural or mechanical ventilation with break resistant windows and either a break resistant two-way mirror or camera that allows for observation of the entire room; (c) a seclusion room may not have locking capability and may not be located in closets, bathrooms, unfurnished areas or other areas not designated as part of residential living space; and (d) a bedroom may not be utilized as a seclusion room and a seclusion room may not be utilized as a bedroom.	☐	☐	☑	☐	☐	This program does not utilize seclusion.
R501-1-9(5) The licensee shall train staff and ensure that the use of any alternate sleeping arrangements other than the client's assigned bedroom complies with Subsection R501-1-4(7) and: **(a)** preserves client dignity and confidentiality; **(b)** shall be done on an individualized, time delimited basis; **(c)** may not be utilized due to staffing shortages or for staff convenience; and **(d)** may not be used as behavior management or consequence.	☐	☑	☐	☐	☑	Program does not have a policy regarding alternative sleeping arrangements and will incorporate it into their Policy and Procedures.
Food Service Requirements	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**

R501-1-10(2) A licensee that provides meals shall: (a) ensure that meals are not used as incentive or punishment; (b) provide nutritional counseling to staff and clients; (c) designate staff responsible for food service who: (i) maintain a current list of each client with special nutritional needs; and (ii) ensure that each client with special nutritional needs has food storage and a preparation area that is not exposed to any identified allergen or contaminant; (d) except in a day treatment program serving clients for less than ten hours a day, or outpatient programs serving clients for less than six consecutive hours a day, provide a variety of three nutritious meals a day that are: (i) served from dietitian or nutritionist approved menus; or (ii) for programs serving individuals experiencing homelessness, serve meals as required by USDA standard homeless settings; (f) provide adequate dining space for clients that is maintained in a clean and safe condition.	☑	☐	☐	☐	☐	
R501-1-10(3) A licensee that allows self-serve meals shall ensure that self-serve kitchen users are supervised, directed, and trained by a staff that has a food handler's permit or is trained by Serv-Safe, USDA, or a comparable program.	☑	☐	☐	☐	☐	
R501-1-10(4) A licensee that serves parents and their children may allow a consenting adult client to maintain full responsibility for their, and their child's, special dietary needs, if consent is maintained in writing in the client record.	☐	☐	☑	☐	☐	Program does not serve parents with children.
R501-1-10(5) A licensee that offers meals for clients shall ensure there is documented training confirming staff are trained to and adhere to the following safe practices: (a) how to identify and accommodate clients with special dietary needs; and (b) allowances for nutritious snacks to be available during restricted hours if the program restricts access to food and kitchen equipment.	☑	☐	☐	☐	☐	
R501-1-10(6) If meals are prepared by clients, the licensee shall inform staff and clients in writing of the following: (a) rules and privileges of kitchen use; (b) menu planning and procedures; (c) sharing self-prepared food; (d) nutrition and sanitation requirements; (e) schedule of responsibilities; and (f) shopping and storage responsibilities.	☐	☐	☑	☐	☐	Meals are not prepared by the clients.
Program Client Record Requirements	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**

R501-1-11(1) The licensee shall maintain client information to include the following: (a) client name, address, email address, phone number, date of birth and identified gender; (b) emergency contact names, including legal guardian where applicable, and at minimum, the emergency contact's physical address, current email address or current phone numbers; (c) a program serving substance use disorder clients shall maintain compliance with an initial and annual client tuberculosis screening results in each client record; (d) any information that could affect health safety or well-being of the client including each medication, allergy, chronic condition or communicable disease; (e) intake screening and assessment; (f) discharge documentation; (g) treatment or service plan; (h) progress notes and services provided with date and signature of staff completing each entry; (i) individualized assessment for restriction of access to on-site items that could be used as weapons, for self-directed violence, or as an intoxicant; (j) any referral arrangements made by the program; (k) client or guardian signed consent or court order of commitment to services in lieu of signed consent for each treatment and non-clinical service; (l) summary of attendance and absences in treatment services; (m) any grievance or complaint made by or against the client and actions taken by the program; (n) each crisis intervention or critical incident report involving the client; and (o) any signed agreement and consent form.	☑	☐	☐	☐	☐	
Program Intake and Discharge Requirements	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-1-12(1) The licensee shall complete an intake screening before accepting a client into the program that includes at least: (a) verification that the client meets the eligibility requirements of the program; (b) verification that the client does not meet any of the exclusionary criteria that the program identified in policy as unable to serve; (c) description of presenting needs; and (d) suicide risk screening.	☐	☑	☐	☑	☑	Program has a pre-screening that covers these items, but it was not completed or not consistently completed in its entirety.
R501-1-12(2) A licensee serving substance use disorder clients may not admit anyone who is unresponsive or unable to consent to care because the individual is experiencing convulsions, in shock, delirium tremens, in a coma, or unconscious.	☑	☐	☐	☐	☐	
R501-1-12(3) A licensee serving incarcerated or court-mandated justice involved clients shall: (a) conduct a criminogenic risk assessment; (c) separate high and low criminogenic risk populations.	☑	☐	☐	☐	☐	

R501-1-12(4) The licensee shall ensure that , the client, parent, or guardian signs and receives copies of the following agreements to be maintained as client records: (a) determination of eligibility; (b) fee agreement outlining costs of services including program, client, parent, or guardian responsibility for payment; and (c) signed consent for treatment that outlines: (i) rules of the program; (ii) expectations of clients, parents, and guardians; (iii) services to be provided; (iv) Medicaid number, insurance information, and identification of any other entities that are billed for the client's services; (v) client rights; and (vi) licensing contact information.	☑	☐	☐	☐	☐	
R501-1-12(5) The licensee shall ensure that a discharge plan identifies resources available to a client and includes: (a) reason for discharge or transfer; (b) aftercare plan; (c) summary of services provided; and (d) progress evaluation.	☑	☐	☐	☐	☐	
Residential Additional Program Intake and Discharge Requirements	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-1-13(1) A residential program licensee shall ensure an intake assessment is completed following an approved intake screening, no later than seven days from the admission date, and that the assessment considers and contains: (a) gender identity and individualized assessment for bedroom and bathroom assignments; (b) cultural background; (c) dominant language and mode of communication; (d) family history and dynamics; (e) current and past health and medical history; (f) social, psychological, developmental, vocational, and, as appropriate, educational factors; (h) authorization to serve and obtain emergency care.	☑	☐	☐	☐	☐	
Program Clinical Services	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-1-14(1) A licensee that offers clinical treatment shall: (a) assign a clinical director to ensure that assessment, treatment, and service planning practices are: (i) regularly reviewed and updated; (ii) individualized; and (iii) designed to involve the participation of each client or each client's parent or guardian; (b) ensure each person working directly with a client is informed of the client's individual treatment needs and advised of the best approach to working with that client; (c) ensure client treatment plans are developed and signed by a licensed clinical professional within 30 days of admission; (d) ensure discharge goals are identified in the initial treatment plan and treatment goals are structured around the identified discharge goals and objectives; (e) ensure that each client identified for treatment receives individual treatment at least weekly; and (f) ensure any missing individual weekly treatment is justified, approved, and documented by the clinical director.	☑	☐	☐	☐	☐	

R501-1-14(2)(a) A residential program licensee shall ensure that in addition to the required weekly individual therapy, frequency and need for family and group therapy and other clinical services are addressed in the individual's treatment plan. (b) A non-residential program licensee who offers clinical treatment may alter the weekly therapy requirement as designated in the individual's treatment plan.	☑	☐	☐	☐	☐	
R501-1-14(4) A licensee who offers group counseling, family counseling, skills development, or other treatment shall offer and document these treatment services as prescribed in the treatment plan.	☑	☐	☐	☐	☐	
Program Staffing	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-1-15(1) The licensee shall ensure adequate staffing to safely supervise the current population, including adding more staff than required by the usual staffing ratio as needed to manage behaviors, dynamics, and individual client treatment and supervision needs.	☑	☐	☐	☐	☐	
R501-1-15(2) The licensee shall identify a manager or qualified designee who is immediately available when the program is in operation or there is a qualified and trained substitute when the manager is absent or unavailable.	☑	☐	☐	☐	☐	
R501-1-15(3) A licensee that offers clinical services shall employ or consult with licensed professional staff that include an individual who is familiar with the program and the needs of each client.	☑	☐	☐	☐	☐	
R501-1-15(4) The licensee shall ensure that before allowing a direct care staff to work unsupervised they have an approved background clearance except as excluded in Section R501-14-17;	☑	☐	☐	☐	☐	
R501-1-15(5) A licensee who serves clients with substance use disorder shall ensure each staff is screened for tuberculosis.	☑	☐	☐	☐	☐	
R501-1-15(6) A licensee who serves a client with substance use disorder may not offer, entice, refer, or recommend medical cannabis as treatment for substance use disorder.	☑	☐	☐	☐	☐	
R501-1-15(7) A licensee who manages, stores, or administers client medication shall identify a medical professional to oversee the medication management, medication oversight, and staff training regarding medication management and administration.	☑	☐	☐	☐	☐	
R501-1-15(8) The licensee shall ensure that each person involved with the prescription, administration, or dispensing of controlled substances maintains appropriate medical or pharmacy licenses and DEA registration numbers	☑	☐	☐	☐	☐	
R501-1-15(9) The licensee shall create and maintain personnel information for each staff member, contracted employee, and volunteer.	☑	☐	☐	☐	☐	
R501-1-15(10) The licensee shall ensure that personnel information includes: (a) any applicable qualification, experience, certification, or license; (b) any approved and current office background clearance, except as excluded in Rule R501-14; (c) a provider code of conduct that is signed by the staff member, contracted employee, or volunteer; (d) any pre-service and annual training records with the date completed, topic, and the individual's signed acknowledgment of training completion; (e) any grievances or complaints made by or against the individual and actions taken by the program; & (f) each crisis intervention or critical incident report involving the individual.	☐	☑	☐	☑	☑	An employee did not have signed acknowledgement of their training completion (only the trainer signed off). Program has a new form that includes the employee signature that they are already using.
R501-1-15(11) The licensee shall ensure that at least one CPR and First Aid-certified staff member is available when staff and clients are present unless a currently licensed healthcare professional is present.	☑	☐	☐	☐	☐	

Personnel Training Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-16(1) The licensee shall ensure that each staff receives pre-serving training on the following topics before being left unsupervised and within 30-days of hire: (a) program policies, procedures and safe practices as outlined in Section R501-1-5; (b) program emergency preparedness, response, and recovery plan, including at least: (i) emergency procedures to instruct staff how to address incident reporting, continuity of care, transport, relocation, and client health and safety during natural disasters, extreme weather events, fire, utility or structural failures, or other unexpected disruptions to the program service; and (ii) instructions to staff regarding how to report and respond to significant criminal activity and significant medical emergencies; (c) CPR and First Aid; (d) client eligibility, emphasizing the behaviors and circumstances the program can safely manage; (e) staff involvement and responsibility in the intake, discharge, and unplanned discharge processes; (f) client rights; (g) supervision and ratios; (h) as applicable, medications management, storing, and administration; (i) as applicable, food handling as outlined in Subsection R501-1-10(3); (j) background checks; (k) prevention, signs and symptoms of abuse and neglect, including sexual abuse, and legal reporting requirements; (l) provider code of conduct as outlined in Rule 380-80; (m) non-discrimination policy in accordance with Section 26B-2-109 that includes a prohibition of abuse, discrimination, and harassment based on sex, gender identity, or sexual orientation; (n) staff and client grievance procedures; (o) crisis intervention; (p) appropriate use of restraint and seclusion; (q) de-escalation techniques; (r) appropriate searches; (s) appropriate and inappropriate behaviors of clients; (t) appropriate and inappropriate staff responses to client behaviors; and (u) if applicable, staff response to a client leaving a program without permission.	☑	☐	☐	☐	☐	

R501-1-16(2) The licensee shall ensure each staff completes the following training topics each year, based on the program's license date: (a) program policies, procedures and safe practices as outlined in Section R501-1-4; (b) general provisions and applicable categorical licensing rule; (c) client eligibility, as outlined in Subsection R501-1-6(1)(e), emphasizing the behaviors and circumstances the program can safely manage; (d) staff involvement and responsibility in the intake, discharge, and unplanned discharge processes; (e) provider code of conduct as outlined in Rule R380-80; (f) program plan for the prevention or control of infectious and communicable disease to include coordination with and following any guidance of the state or local health authorities, Center for Disease Control, and the department; (g) emergency procedures to instruct staff how to address incident reporting, continuity of care, transport, relocation, and client health and safety during natural disasters, extreme weather events, fire, utility or structural failures, or other unexpected disruptions to the program service; (h) program rules regarding firearms that does not conflict with constitutional or statutory rights regarding concealed weapons permits as described in Title 53, Chapter 5, Part 7, Concealed Firearms Act; (i) smoking rules in accordance with Title 26B, Chapter 7, Part 5, Regulation of Smoking, Tobacco Products, and Nicotine Products; (j) how to manage clients who screen with elevated suicide risk levels; (k) general incident reporting; (l) prevention, signs, and symptoms of abuse and neglect, including sexual abuse, and legal reporting requirements; (m) CPR and first aid; (n) if storing and administering medications, training required to administer medication and the process to be followed; (o) training to identify and address in a residential or congregate care program: (i) clients who pose a risk of violence; (ii) what constitutes contraband, possession of contraband, and how the program ensures restriction of client access to contraband and dangerous weapons or materials; (iii) clients who are at risk for suicide; (iv) managing clients with mental health concerns; and (v) identifying the signs and symptoms of clients presenting under the influence of substances or alcohol; (p) if the licensee manages funds for client allowances, training to document each expense; and (q) appropriate use of any alternate sleeping arrangements in a residential or congregate care program.	☑	☐	☐	☐	☐	

Residential Treatment Programs - Inspection Checklist

(Revised Jan 2026)

R = Reviewed.
UR = Under Review. Under Review indicates evidence of potential noncompliance.
NA = Not Assessed during this inspection.

Administration	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
26B-7-117(3) A person that is licensed by the department to provide residential treatment for a substance use disorder shall include as part of the person's admissions materials a question asking whether the individual seeking treatment has ever received services from a syringe exchange program.	☐	☑	☐	☐	☑	This has not been implemented.
R501-19-4(2) Any qualifying residential treatment provider: (a) may not deny any client services solely based on a client receiving medication assisted treatment; and (b) shall ensure any client continues the medication assisted treatment while receiving services from the provider.	☑	☐	☐	☐	☐	
R501-19-4(3) Each residential treatment provider shall ensure its policies include client privacy accommodation in each bedroom space while ensuring client health and safety.	☑	☐	☐	☐	☐	
R501-19-4(9) Each residential treatment provider serving adults may admit a 17-year-old individual if the provider: (a) obtains written permission from the individual's parent or legal guardian; (b) provides clinical justification; (c) ensures that the individual sleeps in a room: (i) separate from any adult or (ii) that the individual shares with one or more adults no more than two years older than the individual; (d) ensures that any adult with direct access to the individual is directly supervised by a direct care staff; and (e) ensures enhanced safety and supervision measures for treating a minor in an adult setting.	☐	☐	☑	☐	☐	Program does not currently have 17 year olds and has not had a 17 year old in the last year.
R501-19-4(10) Each residential treatment provider providing services to a substance use disorder client shall: (a) only admit a substance use disorder client with a level of care that falls within American Society of Addiction Medicine levels 3.1 through 3.5; and (b) obtain any required licenses before providing any service to a substance use disorder client outside of the residential setting with a level of care described in Subsection (8)(a), unless otherwise outlined in categorical rule.	☑	☐	☐	☐	☐	
R501-19-4(11) Each residential treatment provider that allows a client to participate in food preparation shall ensure the client is trained in safe food handling practices and the provider justifies the client's participation in writing.	☐	☐	☑	☐	☐	Clients do not prepare meals.
R501-19-4(12) Each residential treatment provider shall provide individual, group, and family counseling or other treatment, including skills development, at least weekly or as outlined in the individual's treatment plan.	☑	☐	☐	☐	☐	
R501-19-4(13) A clinical professional shall oversee any therapeutic services conducted in the therapeutic environment including: (i) life skill development; (ii) psychoeducation; and (iii) social coaching.	☑	☐	☐	☐	☐	
R501-19-4(14) Each residential treatment provider shall document the time and date of each service provided to each client and include the signature of the individual providing the service.	☑	☐	☐	☐	☐	
R501-19-4(15) Each residential treatment provider shall provide indoor space for free time and informal client activities.	☑	☐	☐	☐	☐	
Specialized Services Required to Serve Clients Under the Division of Services for People with Disabilities	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**

R501-19-6(1) Each residential treatment provider serving a Division of Services for People with Disabilities (DSPD) client shall: (a) apply for any unearned income benefits for which the client is eligible, in conjunction with the support coordinator for DSPD and each client's parent or guardian; (b) develop and adhere to policies and procedures governing the daily operation and activity available and applicable to each client or visitor (c) ensure the facility is located within a reasonable distance from a (i) church (ii) recreation or other community facilities and (iii) school; (d) maintain a record of income and client service fees; (e) maintain a record of each fund deposited with the residential facility for client use; (f) maintain a list of each deposit and withdrawal; (g) maintain a receipt signed by the client and professional staff for any purchase over $20; (h) maintain a record of each client's petty cash fund; (i) present each client with an individual plan that addresses appropriate day treatment; (j) share a monthly activity schedule with each client; and (k) specify, in policy, the amount of time any non-client individual may stay as an overnight guest.	☐	☐	☑	☐	☐	Program is not serving DSPD clients.
R501-19-6(2) If there is a conflict between this rule and the settings final rule, as defined in Rule R501-1, the settings final rule shall prevail.	☐	☐	☑	☐	☐	Program is not serving DSPD clients.

Outpatient Treatment Programs - Inspection Checklist

(Revised Jan 2026)

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NA = Not Assessed during this inspection.

Administration and Direct Services	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-21-4(1) A provider shall: (a) provide general outpatient treatment: (i) on a weekly basis, or (ii) only with individualized clinical justification, less than weekly; (b) only provide intensive outpatient treatment, if offered, for: (i) between nine and 19 hours weekly for adults; or at least six hours weekly for youth; and (c) ensure any client present in the facility for six or more consecutive hours: (i) receives meals and any required medication; and (ii) is placed in the appropriate group size according to building capacity and a physical environment that provides for the comfort of each clients	☑	☐	☐	☐	☐	
R501-21-4(2) (a) An outpatient treatment provider that provides only telehealth services may apply for a single license for one centralized site to cover any telehealth services offered. (b) An outpatient treatment provider that provides only telehealth services shall ensure that any telehealth services provided to an out of state client are done in accordance with the telehealth laws of the client's state of residence.	☐	☐	☑	☐	☐	Telehealth is not typical, outpatient treatment is offered in person.

Substance Use Disorder Treatment Programs	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-21-5(1) Each provider shall: (a) develop and implement a plan on how to support opioid overdose reversal; (b) maintain proof of completion of the National Survey of Substance Abuse Treatment Services annually; and (c) ensure medical cannabis is not used as an enticement for a client or offered, referred, or recommended as treatment for substance use disorder.	☑	☐	☐	☐	☐	
R501-21-5(2) A provider providing medication for opioid use disorder (MOUD) shall: (a) maintain a program-wide ratio of one counselor for every 65 clients to provide adequate substance use counseling to each client as clinically necessary; and (b) ensure that, at least once yearly, each client sees a licensed practitioner that may prescribe controlled substances.	☐	☐	☑	☐	☐	Program does not do MOUD.
R501-21-5(3) Each MOUD provider that prescribes, administers, or dispenses methadone shall: (a) admit a client to the program only after the completion of a face-to-face visit with a licensed practitioner authorized to prescribe controlled substances who confirms that client's opioid dependence; (b) ensure that a licensed practitioner authorized to prescribe controlled substances approves every subsequent dose increase before the change; (c) require each client admitted to the program to participate in random drug testing performed at least eight times a year for each client in maintenance treatment, in accordance with generally accepted clinical practice and with 42 CFR 8 (2025); and (d) require and document at least one hour of prescribing practitioner time at the program site each month for every ten enrolled MOUD clients.	☐	☐	☑	☐	☐	Program does not do MOUD.

R501-21-5(4) Each MOUD program that prescribes, administers or dispenses methadone shall: (a) maintain Substance Abuse and Mental Health Services Administration certification and accreditation as an opioid treatment program; (b) employ: (i) a licensed physician who is an ASAM-certified physician; (ii) a prescribing licensed practitioner who can document specific training in current industry standards regarding methadone treatment for opioid addiction; or (iii) a prescribing licensed practitioner who can document specific training or experience in methadone treatment for opioid addiction; and (c) provide one covered provider, as defined in Section 58-17b-309.7, to dispense or administer medication for every 150 methadone clients dosing on an average of a daily basis.	☐	☐	☑	☐	☐	Program does not do MOUD.
R501-21-6(5) A provider may offer services from a mobile unit, as defined in Section 58-17b-309.7, under the provider's physical site license if: (a) the existing licensed site provides MOUD services; (b) the provider maintains policy and procedures in accordance with department policy applicable to a mobile unit and Rule R523-10; and (c) the provider meets registration requirements of 21 CFR 1300, 1301, and 1304 (April 1, 2021), published by the Office of the Federal Register, incorporated by reference in this rule.	☐	☐	☑	☐	☐	Program is not in a mobile unit.
R501-21-5(6) An alcohol and drug education provider may only provide court-ordered education if certified to do so through the OSUMH in accordance with Rule R523-11.	☐	☐	☑	☐	☐	
R501-21-5(7) A licensed substance use disorder counselor (SUDC) in a substance use disorder outpatient treatment program may: (a) collect client information; (b) conduct the screening portion of an assessment; (c) make level of care recommendations; and (d) identify a substance use disorder.	☐	☐	☑	☐	☐	No clinicians at this program are SUDCs.
R501-21-5(8) A SUDC may not diagnose a client.	☐	☐	☑	☐	☐	No clinicians at this program are SUDCs.
R501-21-5(9) A OSUMH-certified alcohol and drug education provider shall: (a) complete and maintain a substance use screening: (i) for each client before providing the education course; and (ii) that may be shared between providers with written client consent; (b) provide a workbook to each client to keep upon completion of the course; (c) ensure at least 16 hours of course education; and (d) provide separate classes for adults and youth.	☐	☐	☑	☐	☐	Program does not provide OSUMH certified education.
R501-21-5(10) A provider offering services to a justice-involved client shall: (a) operate in compliance with Rule R523-3; (b) maintain a validated criminogenic screen or risk assessment for each justice-involved client which is conducted with an accepted tool, including: (i) Level of Service Inventory-Revised (LSI-R); (ii) Risk and Needs Triage (RANT); (iii) Ohio Risk Assessment System (ORAS): or (iv) any other screen that the provider can demonstrate the validation to the OSUMH: (c) separate clients into treatment groups according to each client's assessed level of risk; (d) complete screenings that assess substance abuse and mental health comorbidity; and (e) treat, or refer to another licensed department program that serves justice-involved clients to treat, an array of disorders noted in the screening.	☑	☐	☐	☐	☐	
Domestic Violence Programs	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-21-6(1) A domestic violence (DV) treatment provider shall comply with generally accepted and current practices in DV treatment.	☐	☐	☑	☐	☐	Program is not a domestic violence program.
R501-21-6(2) A DV treatment provider shall: (a) maintain and document cooperative working relationships with any DV shelter, treatment program, referring agency, local DV coalition, and custodial parent when the client is a minor; (b) offer at least ten sessions of treatment for any child or victim, not including intake or orientation; (c) if the client is a perpetrator, contact any victim, current partner, or required criminal justice referring agency; and (d) ensure a licensed mental health therapist, as defined in Section 58-60-102, completes a DV treatment evaluation for each client who is a perpetrator that includes individualized recommendations for that client's treatment.	☐	☐	☑	☐	☐	Program is not a domestic violence program.

R501-21-6(3) For groups, a provider shall ensure there is at least one staff for: (a) there is at least one staff for every eight clients in an adult treatment group for up to an hour; (b) every ten clients in an adult treatment group exceeding one hour; (c) every eight children in a child victim or child witness group where each child client is under 12 years old; and (d) every ten children in a child victim or child witness group where each child client is at least 12 years old.	☐	☐	☑	☐	☐	Program is not a domestic violence program.
R501-21-6(4) For client safety, the provider shall: (a) ensure that: (i) when a client enters a DV treatment program, the staff conduct an in-depth, face-to-face interview and intake assessment to determine the client's clinical profile and treatment needs; and (ii) when the client is a perpetrator, the evaluation described in Subsection (2)(d) ; (b) obtain additional information for any client who is a perpetrator from any police incident report, criminal history of the client, prior treatment provider, victim, or victim advocate; (c) when appropriate, obtain additional information for a child client from any parent, prior treatment provider, school, or the Division of Child and Family Services Child Protective Services; (d) when any document listed in Subsections (4)(a) through (c) cannot be obtained, document the reason; and (e) ensure that the intake assessment or evaluation described in Subsection (2)(d) includes: (i) a profile of the frequency, severity, and duration of the DV behavior, including a summary of any psychological violence; (ii) documentation of any homicidal, suicidal ideation and intentions, or abusive behavior toward children; (iii) a clinical diagnosis and a referral for evaluation to determine the need for medication, if indicated; (iv) documentation of safety planning when: (A) the client is an adult victim, child victim, or child witness; and (B) that client has contact with the perpetrator; (v) documentation of a safety plan for any victim who chooses not to become a treatment client; and (vi) documentation that appropriate measures have been taken to protect children from harm.	☐	☐	☑	☐	☐	Program is not a domestic violence program.
R501-21-6(5) For treatment procedures,a provider shall: (a) create an individualized treatment plan addressing relevant treatment issues for each client; (b) refer each client deemed not appropriate for a DV program to the appropriate resource or program, with any reason for referral documented, and notify the referred agency; (c) provide DV counseling concurrently with, or after, other necessary treatment when appropriate; (d) ensure that any client who is a perpetrator, or a co-perpetrator, is not provided conjoint or group therapy in the same session as any victim who has experienced violence from the perpetrator or co-perpetratoruntil staff has completed a comprehensive assessment to determine that: (i) any violence between the client who is a perpetrator, or a co-perpetrator, and the victim has stopped; and (ii) conjoint therapy is appropriate; (e) require the client who is a perpetrator to complete at least four DV treatment sessions, unless otherwise noted in the evaluation recommendations described in Subsection (2)(d), before the provider implements conjoint therapy; (f) implement written procedure in an efficient and timely manner to facilitate any: (i) entry of a court-ordered defendant into treatment as a client; (ii) notification of client compliance, participation, or completion; (iii) disposition of a non-compliant client; (iv) notification of the recurrence of violence; and (v) notification of factors that may exacerbate a client's potential for violence; (g) comply with the duty to warn; (h) document specialized training in DV assessment and treatment practices for any individual providing treatment service, including: (i) completing 24 hours of Utah Association for Domestic Violence Treatment pre-service training; (ii) completing 16 hours annual training after the initial training in Subsection (5)(h)(i); and (iii) at least one hour per week of clinical supervision for any treatment staff that is not clinically licensed to discuss clinical dynamics of a case.	☐	☐	☑	☐	☐	Program is not a domestic violence program.
R501-21-6(6) The provider shall ensure training is documented and approved by the designated specialist regarding assessment and treatment practices for treating DV victims and perpetrators.	☐	☐	☑	☐	☐	Program is not a domestic violence program.
R501-21-6(7) A provider shall disclose any: (a) current department contracts; or (b) action against the contract to the Office of Licensing.	☐	☐	☑	☐	☐	Program is not a domestic violence program.
R501-21-6(8) A provider shall disclose any current accreditation or action against accredited status to the Office of Licensing.	☐	☐	☑	☐	☐	Program is not a domestic violence program.

Social Detoxification Programs - Inspection Checklist

(Revised Jan 2026)

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UR = Under Review. Under Review indicates evidence of potential noncompliance.
NA = Not Assessed during this inspection.

Physical Facility	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-11-5(2) Each provider shall ensure that the physical facility is compliant with the following: (a) 24 hour live-in staff have a separate living space with a private bathroom; (b) a large room may be used as a dormitory style bedroom; (c) a minimum of 50 sq ft per individual, excluding storage space, is provided for a multiple occupant bedroom; (d) a minimum of 70 sq ft per individual, excluding storage space is provided for a single occupant bedroom; (e) there is an escape window for each sleeping room unless there are two ways to exit the room; & (f) each bathroom meets a minimum ratio of one toilet, one sink, and one tub or shower for each eight residents.	☑	☐	☐	☐	☐	

Specialized Services	**R**	**UR**	**NA**	**Corrected During Inspection**	**Technical Assistance Given**	**Notes**
R501-11-6(1) Social detoxification services shall serve clients who require a clinical managed residential withdrawal management level of care only until they can be stabilized and transition to a lower level of care, or medical necessity requires moving the individual to a higher level of care.	☑	☐	☐	☐	☐	
R501-11-6(2) A provider wishing to provide medically monitored inpatient withdrawal management under a social detoxification license may only do so under the following conditions: (a) medical and nursing professionals provide 24-hour medically monitored evaluation and withdrawal management under physician-approved policies and physician-monitored procedures and protocols; (b) justification is documented for how clients served at this level do not require services at the level of either a higher or lower level of care; (c) the individual meets the admission and continued service criteria for medically monitored withdrawal management; and (d) the program meets each requirement for medically monitored withdrawal management.	☑	☐	☐	☐	☐	
R501-11-6(3) The provider shall complete a preliminary screening when an individual presents for service to determine appropriateness for social model detox and shall complete the subsequent intake evaluation within seven days.	☑	☐	☐	☐	☐	
R501-11-6(4) The provider shall require a client to provide recent evidence of a Tuberculosis screening or get tested for Tuberculosis within one week of presenting for service.	☑	☐	☐	☐	☐	
R501-11-6(5) The provider shall ensure: (a) a client who exhibits signs of possible active Tuberculosis is screened by the provider immediately with assistance from the local health department; and (b) any local health department recommendations are followed.	☑	☐	☐	☐	☐	
R501-11-6(6) Once a client has completed the acute detoxification period as demonstrated by reasonable physical and psychological stability, the provider shall conduct an evaluation to determine the treatment referral.	☑	☐	☐	☐	☐	